

METROPOLITAN NEUROLOGICAL INSTITUTE

Department of Neurodiagnostics & Clinical Neurophysiology

PATIENT: Jane Doe DOB: 10/14/1978 ID: NX-884920
EXAM DATE: October 08, 2026 REFERRING: Dr. S. Vance, MD
PROCEDURE: MRI Brain Without Contrast

CLINICAL INDICATION

48-year-old female presenting with episodic tension-type headaches and mild dizziness. Rule out intracranial pathology or mass lesion.

TECHNIQUE

Multiplanar, multisequence magnetic resonance imaging of the brain was performed on a 3.0 Tesla scanner including sagittal T1, axial T2, axial FLAIR, diffusion-weighted imaging (DWI), and ADC maps without intravenous gadolinium contrast.

FINDINGS

BRAIN PARENCHYMA: The cerebral hemispheres demonstrate normal symmetric cortical gyral architecture. There are a few scattered punctate foci of high T2/FLAIR signal intensity in the subcortical and deep periventricular white matter of both frontal lobes. No diffusion restriction on DWI to suggest acute territorial infarction. No evidence of intracranial hemorrhage or mass effect. Midline structures are centered.

VENTRICULAR SYSTEM: The lateral, third, and fourth ventricles are normal in size and symmetric for age. Basal cisterns are clear.

EXTRA-AXIAL SPACES: No extra-axial fluid collections or subdural hematoma.

PARANASAL SINUSES & ORBITS: Mild mucosal thickening noted in the bilateral maxillary sinuses and ethmoid air cells. Mastoid air cells are well-aerated.

IMPRESSION

1. No acute intracranial hemorrhage, mass effect, or territorial infarction.
2. Minimal scattered punctate T2/FLAIR white matter hyperintensities in the frontal lobes, non-specific, most commonly representing chronic microvascular ischemic changes or migraine-related changes.
3. Mild incidental bilateral maxillary sinus mucosal thickening.

Electronically signed by: Marcus Sterling, MD, Neurologist / Clinical Neurophysiologist
Board Certified in Neurology and Electrodiagnostic Medicine